

# Side Rail Evaluation Flowchart – Skilled Nursing Facility

---

## Resident Admission or Change in Condition



## Therapy Assessment (PT / OT)

- Assess bed mobility (rolling right/left, supine ↔ sit)
- Upper extremity strength and trunk control
- Cognition and safety awareness
- Ability to use side rail safely and independently



## Purpose

Side rails are considered only for functional mobility, including:

- Bed mobility assistance
- Repositioning
- Supine to sit transitions

Not for restraint or fall prevention alone.

## Side Rails Are NOT Used For

- Fall prevention
- Containment
- Behavior management
- Staff convenience



## Least Restrictive Options Reviewed

Consider and document alternatives:

- Low bed
- Landing mat
- Trapeze
- Transfer pole
- Increased supervision



## Decision

Clinically indicated and least restrictive?

If NO:

- Document: Side rails not indicated
- Recommend alternatives
- Communicate to nursing

If YES:

## Therapy Documentation

- Clinical rationale
- Type of side rail recommended (1 or 2 side rails)
- Resident ability to use safely
- Alternatives considered
- Clarify that use is not intended as a restraint

## Communication

- Recommendation communicated to nursing
- Documented in therapy note

## Therapy Note Verbiage

If Side Rails Are Recommended:

Resident requires side rail for functional bed mobility and repositioning. Resident able to use safely and independently. Least restrictive alternatives considered. Use is not intended as a restraint. Recommendation communicated to nursing.

If Side Rails Are NOT Recommended:

Side rails not indicated at this time due to inability to use safely. Alternatives recommended. Recommendation communicated to nursing.